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THE '*DERMOGROTESQUE*' BODY: ACNE, ALIENATION, AND AESTHETICS IN BUKOWSKI'S *HAM ON RYE*

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ABSTRACT
This article examines Charles Bukowski's *Ham on Rye* through the proposed framework of the Dermogrotesque which positions the skin as the primary site where social, medical, and aesthetic meanings intersect and are inscribed. By focusing on Chinaski's acne, the analysis draws on Foucault, Siebers, and other critical theories of embodiment to trace how visibility becomes a conduit for shame, exclusion, and epistemic violence, situating Bukowski's work within conversations on disability, aesthetic capitalism, and the socio-medical gaze. The study argues that the novel's episodic, anti-teleological structure is inseparable from its dermogrotesque embodiment. Chinaski's skin cannot be narratively redeemed, and Bukowski refuses the coherence, transformation, or moral clarity often expected of illness narratives. He produces a form of narrative that is ethically and formally consistent with chronic stigma – fragmented, flat, and unresolved. This reading demonstrates how *Ham on Rye* contributes to medical humanities by revealing the narrative and affective implications of living in a grotesquely visible skin.

Introduction

> "The personal issue of telling stories about illness is to give voice to the body, so that the changed body can become once again familiar in these stories. But as the language of the story seeks to make the familiar, the body eludes language." (pg. 2)
>
> Arthur Frank, 1995

The difficulty of articulating an altered body becomes, in Charles Bukowski's *Ham on Rye*, a narrative force. Acne emerges as a central, ingrained crisis in the protagonist Henry Chinaski's life. It is unwanted, much like Chinaski's marginalized social class, yet it persists and declares itself unapologetically, making the "self" *invisible* and the "social" *profound*. Writings of the social, cultural, and political are superimposed on the biological and the physical. Such insistence on the marked and suffering body resonates closely with Bukowski's own adolescence. Barry Miles writes, "Becoming a man caused Hank [Bukowski] even more distress" because of the painful acne and boils that covered his skin (29).[1] His endless tryst with doctors and hospitals (in his adolescence for acne and later years for other illnesses), along with the social (and academic) ridicule, shaped his sense of visibility, vulnerability, and estrangement, which his alter ego Chinaski also carries (Cherkovski 32–33). Oliver Harris, in his review of Ham on Rye, stated, "Bukowski's own ugliness is vital to the etiology of his pain, which in turn shapes the acerbic disgust and disinterested grace of his prose," (65) indicating that early readers and critics already understood his self-figured grotesqueness as inseparable from his literary style.

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It is within this entanglement of corporeal pain, social scrutiny, and narrative complexity that this article situates its intervention. It examines Bukowski's portrayal of acne as a powerful critique of medical pathologization and aesthetic capitalism, highlighting how Chinaski's skin becomes a site of surveillance, regulation, and moral examination. To analyze this, the article proposes the **Dermogrotesque** as a conceptual lens to name a focused configuration of grotesque embodiment in which the skin – the body's visible and most policed boundary – becomes the primary site for the manifestation of the grotesque, functioning as a permeable interface that absorbs socio-medical, aesthetic, and capitalist inscriptions while expressing internal states such as shame, resistance, and alienation. As a mode of reading, dermogrotesque highlights the skin's hypervisibility, enforced invisibility, permeability, ambivalence, and social inscription that structure both Chinaski's experiences and the novel's narrative logic. This concept draws upon the major theories of the grotesque body and expands it to consider the specific dermatological context, informed by Foucault's theory of medical gaze and biopower and Siebers' work on aesthetic disability. Rosemarie Garland-Thomson asserts, "The extraordinary bodies," which are the *dis-abled* bodies, "reside not in inherent flaws, but in social relationships," (*Extraordinary Bodies* 7) which makes them a text upon which cultural meanings are inscribed. Similarly, Couser notes that life writing about illness "has come of age" (7), serving as both a cultural and political act: "representative in two senses of the term: aesthetic (mimetic) and political (acting on behalf of)" (3). In *Ham on Rye*, Chinaski's acne becomes that inscribed surface, narrating personal suffering but also exerting resistance against dominant medical and aesthetic discourses.

The article proceeds in a layered analysis by first discussing the dermogrotesque as a conceptual lens and situating Ham on Rye within the broader category of illness narratives. Second, it conducts a close reading of Chinaski's dermogrotesque flesh, tracing the grotesque inscription of acne on the skin. Third, it examines the capitalist and aesthetic dimensions of surveillance, shame, and the skin economy as expressed within the novel. Finally, it closes with a discussion around the body as a narrative surface and reading acne as an illness narrative. Together, these analyses illuminate acne's literary role as both a wound and a warning, where it becomes a sign of deviance, a site of control, and a skin-deep indictment of medical and cultural power structures.

The Dermogrotesque as a Conceptual Framework

As a conceptual lens, the dermogrotesque designates a mode of reading in which the skin becomes the primary surface of grotesque inscription. Unlike broader formulations of the grotesque in Bakhtin (687–692) or the abject in Kristeva (2–8), this lens narrows its focus to the dermatological interface where visibility, vulnerability, and social meanings intersect. The dermogrotesque therefore does not simply evoke bodily excess, carnivalesque distortion, or symbolic breakdown; it examines and emphasizes how the *skin* becomes hypervisible or forcibly invisible and a permeable surface for socio-medical violence, aesthetic judgment, and existential alienation.

Central to this lens is the bidirectional flow of meaning across the skin's surface: it *absorbs* external gazes (medical, social, aesthetic, and capitalist) while *projecting* internal states (shame, resistance, and affective instability). The grotesque emerges through this continual exchange. It is activated not by spectacular deformity alone but also by the skin's susceptibility to scrutiny, misreading, and normalization. In this way, the dermogrotesque resides at the intersection of ableist expectations, aesthetic capitalism, and moralized narratives of bodily discipline. This framework retains the affective duality of grotesque – the horror and the humor – yet departs from both Bakhtinian carnivalesque incoherence and Kristevian symbolic collapse. Instead of collapsing order, the dermogrotesque refuses narrative coherence in embodied form, where the unstable skin surface blurs distinctions between human and abject, healthy and diseased, and visible and invisible. It generates the layered affective responses characteristic of grotesque embodiment (revulsion, pity, and dark humor) while anchoring them in specific dermatological experience. The grotesque is activated and amplified by its hypervisibility under intersecting social, medical, and aesthetic gazes. These gazes pathologize difference

(Foucault's medical gaze), enforce normative beauty standards (Siebers' aesthetic disability), and often inscribe judgments related to class, morality, and self-discipline onto the visible skin. Dermogrotesque is less focused on the psychoanalytic breakdown of symbolic order and more centered on the socio-cultural and aesthetic inscription onto the visible surface and the role of the external gaze. In this sense, the dermogrotesque offers a focused, skin-centered vocabulary for understanding the grotesque embodiment's production, reading, and resistance.

Ham on Rye as an Illness (Non)Narrative In-Tension

Ham on Rye can be read not only through the grotesque surface of Chinaski's body but also through the pressures this surface exerts on narrative form. Chinaski's acne, his primary dermogrotesque condition, refuses coherence at both the level of embodiment and storytelling. The instability, ambivalence, and affective volatility that mark the dermogrotesque also shape the novel's formal structure, producing a narrative that resists the conventions of illness narratives. Thus, the novel occupies a space of tension between narrative and non-narrative, staging acne as an event that the text cannot assimilate into a developmental arc, redemption, or meaning-making. This tension allows Ham on Rye to serve as a powerful site for interrogating the limits, ethics, and possible ideological assumptions of illness narratives.

While medical/illness/health Humanities have broadened their scope, the field has historically privileged chronic conditions – cancer, HIV, autoimmune illness – treated as archetypal narrative forms organized around disruption and coherence (Frank, *The Wounded Storyteller* 119). Critical medical humanities critique this narrowness, urging attention to broader social and political "entanglements" (Viney et al. 2–7) and "geographies" (Atkinson et al. 73–75). Reading acne as a *wounded script* foregrounds the communicative force of pain, marginalization, and aesthetic regulation.

Although *Ham on Rye* is often read as a bildungsroman, its acne plot produces an illness narrative structurally and ethically at odds with that genre. Scholars such as Strawson (198; 428), Woods (74–76), and Bolaki (12) challenge narrative essentialism that plagues illness narratives, arguing that narrative coherence is neither universal nor inherently therapeutic. Bukowski's text dramatizes this critique: Chinaski recounts his experience without offering moral growth, insight, or universality. His acne, abuse, and alienation appear not as metaphors but as facts – "mostly fact but also adorned with a bit of fiction," as Bukowski remarked in 1975 (Calonne 125). Bigna (89–107) notes that Bukowski's writings often confound conventional critical tools, as examining shifts in narrative style can illuminate his unique approach to representing life experiences. The novel's refusal to redeem Chinaski or convert pain into meaning destabilizes readerly expectations and resists the narrative conventions of character development and emotional closure. This flatness is not formlessness but another mode of narrative rhythm – episodic, repetitive, and tonally inconsistent. The institutions that shape Chinaski's life – family, school, religion – attempt to impose coherence through discipline, shame, and moralizing narratives. Acne becomes intelligible only as punishment or personal failure, turning him into a freak (Charlson 62–65). Meulenberg and Beaufort (351–52) identify a comparable logic of clinical spectacle and patient objectification in Bukowski, albeit in relation to his depiction of hemorrhoids. Such imposed coherence constitutes narrative violence, intensifying rather than alleviating suffering. Bukowski counters this by refusing climax, redemption, or symbolic elevation: Chinaski's acne does not produce wisdom; his abuse does not yield empathy. Instead, the novel witnesses rather than explains trauma, aligning with trauma theory's insistence that trauma resists narrative closure.

While it is a narrative, it undermines the assumptions typically attached to life stories: growth, meaning, transformation. Instead, it offers a narrative that functions more as witnessing than explaining. Chinaski's story is not designed to integrate trauma into a coherent self, but to present the fractured experience of a self, shaped by trauma. In this sense, Bukowski anticipates contemporary trauma theory, which holds that trauma often resists narrative closure or coherence. Still, the very act of telling – even in flat, broken prose – becomes a form of narrative restitution. Bukowski thus occupies a space between the pro-narratives and anti-narratives: he neither embraces full narrative

identity nor fully abandons narrative as a mode of representation. Malone (8) writes of Bukowski's writing, "He usually disappoints [on this score], preferring to end [them] on an anticlimactic dying fall." In the novel, he employs a stark visual and spatial vocabulary that disrupts traditional narrative thought and even challenges Strawson's anti-narrative model. Recurring spaces like bathrooms, classrooms, and alleyways are not metaphorical but viscerally real, filled with blood, pus, sweat, and humiliation. These are zones where the body is unprotected, and identity is reduced to mere physicality. Rather than helping the protagonist develop or evolve, these spaces enforce stasis, isolation, and numbness. Similarly, visual motifs like mirrors reflect not self-awareness but grotesque self-alienation. The environment itself resists symbolic coherence, emphasizing that meaning is not imposed from above but erodes from below. The novel thus challenges the pro-narrative belief that coherence is integral to self-understanding, while also complicating Strawson's claim that non-narrative selves lack narrative value. *Ham on Rye* presents a life that is episodic, anti-heroic, and resistant to integration, yet still narratively powerful. In its refusal to redeem, explain, or console, Ham on Rye exposes the limits and the ethical risks of coherence itself. It reveals how narrative can both expose and resist harm, and how trauma may persist without being shaped into a developmental arc. Acne becomes a rare literary space where trauma is not interpreted but allowed to persist and be seen.

Chinaski's Dermogrotesque Embodiment

If the previous section established the novel as an illness (non)narrative, then the next step is to turn to Chinaski's body itself on which this narrative tension is inscribed. Reading Chinaski's acne through the dermogrotesque reveals how his skin becomes the novel's most active narrative agent – a profound literary device that allegorizes social rejection, existential despair, and metaphysical abandonment. It becomes a narrative mapping of societal cruelty onto the protagonist's body.

Chinaski's acne-ridden flesh is marked by the doubleness of horror and humor, the hybridity that violates bodily boundaries and the metamorphosis that resists stable categorization. The lived and unlived, the visible and invisible, the self and other are inscribed onto his flesh. When Chinaski observes, "The boys in the cars didn't worry about acne" (126) Bukowski constructs a visceral class critique through epidermal contrast.[2] The homemade shirt, rundown shoes and pants, become a sartorial extension of his dermatological abjection, the visible markers of his "50-cents-a-week" (155) existence on his skin. This epidermal class inscription sutures economic precarity directly to appearance, transforming poverty into a dermatological condition. This abjection not only constrains his opportunities and social mobility but also builds his sense of insignificance and powerlessness in society. Chinaski's alienation intensifies his perception of invisibility. He faces difficulty in forging meaningful connections with others due to constant bullying and ridicule and often feels like he is at the periphery. Acne makes it even more challenging for him to approach and interact with girls, further deepening his self-perception as a marginalized figure. At one point in the novel, he refers to himself as a "Jungle animal" (193) because his face is not merely blemished; it is abjectified, rendered a site of social horror. It's horrid not because it threatens others, but because it makes disorder, suffering and imperfection visible. In line with Frank's concept of "the wounded storyteller" (xvi), Chinaski is the sufferer and witness whose grotesque skin becomes an involuntary narrative of pain and resistance. His body becomes a visual transgression against norms of beauty, cleanliness, and coherence.

Religious violence also compounds his abjection and grotesquerie. Chinaski's grandmother viewing his sores as signs of demonic possession, attempting to exorcise "the devil from his body" and jabbing a crucifix into his back, "making a hole for the devil to be pushed out by God" (139–140) not only invoke horror but also a certain degree of humor. The grandmother's exorcistic violence literalizes the dermogrotesque: the grotesque skin is breached as if to purge moral and metaphysical impurity. Chinaski's claim that "God has forsaken" (142) him also echoes this metaphysical abandonment. Acne provokes laughter and revulsion simultaneously; it blurs the line between human and abject form; and it refuses coherent identity, marking Chinaski as perpetually liminal. His skin, full of lesions, cysts,

boils the size of walnuts (127) disturbs identity, system, order (Kristeva 4) of the medical, aesthetic and metaphysical.

The dynamic is central to the dermogrotesque which locates the grotesque not simply in bodily distortion but in the power-laden gaze that renders the skin hypervisible and theatrically diagnostic. Michel Foucault's *medical gaze* exposes how dermatology reduces Henry Chinaski's acne to a spectacle of pathology in *Ham on Rye*—a dehumanizing act that mirrors the novel's broader interrogation of illness narratives. As Smith and Watson argue, autobiographical acts reconstruct lived experience through language (32), and Bukowski's semi-autobiographical account operates as what Arthur Frank terms a "chaos narrative": one that resists redemptive arcs to document the fragmentation of self through bodily stigma (97–100). The medical gaze enacts this fragmentation by transforming the sufferer from an integrated subject into a dissectible object, as Foucault asserts: "the gaze is no longer reductive, it is, rather, that which establishes the individual in his irreducible quality" (*Birth of the clinic* xiv). This epistemological violence is literalized when dermatologists performatively diagnose Chinaski's acne – "The worst case I've seen! Fantastic! Look at the face!" (131) – reducing him to a "*case*" in a capitalist hierarchy that ties worth to bodily restorability. This exclamation is not merely diagnostic; it is theatrical, intended for an audience of other doctors who treat Chinaski as pathology incarnate. Diagnosis here is not an act of *recognition* but *reinforcement* of classificatory hierarchies.

Chinaski's body becomes a text to be read by institutional authority, subjected to experimental treatments like hot light and electric needles (134). He is "drilled" mechanically and when he "gives no sign at all," becomes a spectacle of unwanted resistance (134–137). His humanity is erased beneath layers of interpretive signs and authority. The clinic abstracts the patient in the name of truth (Foucault, *Birth of the clinic* 8) through a hierarchy of medical discourse that silences Chinaski's subjectivity: "You really have a case there, haven't you?" (133). Nurse Ackerman also voices this silencing: "They all quiet down, finally" (143). This rhetorical quieting marks the patient's transformation from speaking subject to compliant object, the ideal patient of the medical regime. The doctor's examination of Chinaski's acne emphasize the detachment of the subjective: "He turned a hot white light on me and looked me over . . . He kept looking . . . Some moments passed and suddenly there were many people in the room. They were all doctors . . . " (131). The doctor who had "finished examining a young girl with acne vulgaris" mocks that she cried and told him: "How will I ever get a man? . . . I want to kill myself!' And now look at this fellow! If she could see him, she'd know that she really had nothing to complain about!" (131). In this moment, Chinaski is reduced to a cautionary exhibit, a didactic object in the medical theater of suffering and clinical comparison. His acne becomes an aesthetic disability that violates aesthetic norms; dermatologists reduce his suffering to a "fantastic" spectacle (131), enacting the violence of bodily perfection. Even sympathy from Miss Ackerman feels strange because his body has already been made strange (as A. Frank would put it) as a spectacle of amusement, due to which sympathy feels unsettling as it doesn't fit in the typical detached medical (because she's a nurse) and human (since everyone mocked him) observation.

This process is where *seeing* becomes *a way of knowing* to support the claim to understand the truth of the body (Foucault, *Birth of the clinic* 120–21). Bukowski's critique is sharpest here: the medical institution does not offer redemption or cure but displays Chinaski as a *case-in-condemnation*.

The silencing extends beyond the clinic. Chinaski's acne attracts the "staring" gaze that Rosemarie Garland-Thomson identifies as an "interrogative gesture" that intrudes upon the subject of privacy and demands explanation (*Staring* 3). This stare further haunts Chinaski in social spaces, where acne renders him "an audience of one" (122), alienated from peers who perform normality on a stage he cannot join. The *gaze* when mixed with the *stare*, produces a socio-medical gaze that is never innocent but embedded in symbolic violence that dictates who belongs and who deviates. His impoverished background, homemade shirts and hand-me-down shoes are already visible markers of his social exclusion. Acne, as a hyper-visible extension of this marginalization, functions as both symptom and signifier. It renders the inner trauma legible, inscribing it on the body's surface in a manner that culture is quick to read and condemn. As Stagno and Blackie emphasize in their discussion of Bukowski's short story "*The most beautiful woman in town*", it is "the sinking feeling of overlooking

a seemingly minor detail whose significance only becomes apparent too late," (121–23), a dynamic mirrored in both Cass's scars and Chinaski's acne, as they are read – or misread – by peers, medical professionals and society at large.

Chinaski avoids mirrors, often "wondering how ugly a person could get" (137), turns away from others' gazes, and eventually internalizes the revulsion he inspires. He prefers lying in bed all day because then he won't have to see anybody: this retreat from the socio-medical gaze depicts how grotesque flesh restructures space and self-perception. His body becomes a spectacle even for himself, which entraps and displays him simultaneously. At one point, after undergoing medical treatment, he recounts: "My whole head was bandaged. I was all white. Nothing could be seen but my eyes, my mouth and my ears, and some tufts of hair sticking up at the top of my head. I was hidden. It was wonderful . . . I hoped I could stay this way forever" (144). The bandage not only serves as a medium to heal but also as a source of escape from the socio-medical gaze. It brings about a certain mystery about him, thus amplifying the grotesque while simultaneously hiding it, often becoming a medium for his resistance against the socio-medical gaze and aesthetic disability.[3] This self-invoked invisibility subverts the clinical ideal of visibility, which makes him hyper-aware of his own embodiment, no longer able to inhabit his body unselfconsciously.

The socio-medical gaze aligns with Tobin Sieber's theory of aesthetic disability in which aesthetic judgments about bodies – smoothness, symmetry, "normal" beauty – operate as powerful social exclusions (71). These visible differences disrupt normative beauty. Chinaski's acne is framed as a *classed* failure. Siebers critiques dermatology's fixation on *cure* over accommodation; for Chinaski, this manifests as unaffordable treatments and workplace stigma, reinforcing what Jasbir Puar calls debility – the systemic neglect of "unfixable" bodies (xiii-xiv). What cannot be aestheticized must be pathologized. Foucault's question: "by what right can one transform into an object of clinical observation a patient . . . " (*Birth of the clinic* 83) finds its counterpart in Siebers' insistence that aesthetic disability marks the body as socially undesirable (71–2). The youthful male face, which should represent vigor and promise, instead starts connoting contagion and shame when disfigured. Acne, under the socio-medical gaze, is treated not because it causes physical harm, but because it offends the aesthetic order. The socio-medical gaze is internalized into unworthiness and further becomes the "shaming" gaze (Dolezal ix-x). Chinaski's suffering is thus twofold: he is both the object of medical inspection and the bearer of aesthetic deviance. Acne, in this narrative, is an invitation to expulsion because what is seen by the gaze and what escapes, comes into or escapes existence (Foucault, *Birth of the clinic* 96). This also reinforces Sieber's contention that aesthetic differences are not merely seen but structurally oppressed. Thus, Chinaski's acne becomes a social verdict before it becomes a medical diagnosis.

Shame dictates his choices as he exclaims, "I was ashamed of my boils at school. At Chelsey, you had a choice between gym and ROTC. I took ROTC because then I didn't have to wear a gym suit, and nobody could see the boils on my body" (126). This shame is compounded by parental disappointment, illustrated when his father who had been trying to heal his acne using a pungent and noxious brown salve, expressed regret over having a son like him, which the protagonist believes is due to his repugnant appearance. He perceives his father as a manifestation of everything he despises about the world, encompassing societal norms and expectations that suffocate his individuality – to some extent like the non-existent God. Chinaski offers his father occasional sympathy despite his stark dislike of him, which he doesn't to God. This ambiguity is established again during the hallucination scene, where the non-existent faceless God against whom Chinaski contemplates violence, metamorphizes into a dark demonic entity that incites fear and speechlessness. Here, the grotesque proliferates not on the skin alone but also in the psychic residue it leaves behind.

Bukowski captures the paradox of Chinaski's embodiment: "[Outside of that,] I had a good body. But nobody would notice that" (168). His fit torso is erased by the hypervisibility of his acne, affirming that staring collapses the person into their anomaly. Only the "defect" is legible; all other qualities are effaced. The body, instead of facilitating social engagement, becomes a site of exile. In this sense, acne is architectural, structuring Chinaski's movements, limitations, and worldview.

Bukowski's style – brutal, unsentimental – amplifies this existential horror. Sentences like "I was the worst case in town . . . I had to withdraw. I watched people from afar; it was like a stage play" expose the raw brutality of living in a grotesque body. This body quite often becomes an experimental site, where the poor are tested and discarded: "If it didn't work, there would still be more needy people to test it on" (136). This is how institutional regimes like medicine extend their control and surveillance over bodies through discourses of care, discipline and experimentation (Foucault, *Birth of the clinic* 48–72). Bukowski links material poverty to medical disposability, merging class critique with the dermogrotesque. This institutionalized marginalization extends into Chinaski's very sense of self, producing epistemic and ontological violence – a fundamental denial of agency and recognition. Through grotesque embodiment in the novel, Bukowski refuses both aestheticization and transcendence. Instead, he confronts readers with the unrelenting horror of living inside a body that society is eager to erase yet compulsively stares at. If the medical and aesthetic gaze renders Chinaski's body a site of pathology and exclusion, Bukowski's narrative intensifies this alienation by staging acne as "grotesque" flesh. Chinaski's severe acne and his skin become a spectacle, wound, text and a permeable boundary. Within this dermogrotesque framework, acne becomes destiny, inscribed onto the flesh with the tools of surveillance, shame, and economy of the skin.

Surveillance, Shame, and the Skin Economy

Chinaski's acne moves beyond the space of medical intervention and socio-cultural marginalization under a dermogrotesque reading. It lies also in the ideological space of aesthetic capitalism where it is a defect to be hidden, corrected, or commodified. His disfigured face, which isolates him from peers and romantic possibilities, exists in direct opposition to the sleek, flawless surfaces celebrated in commercial imagery. His condition becomes an impediment not just to health but to desirability, marketability, and social legibility. He becomes illegible within a visual economy that prizes clarity, smoothness, and discipline.[4] His body is thus, doubly marked as excessive in its visible eruptions and as deficient in its failures to conform. Acne is not simply dermatological here; it becomes a failed commodity.

The cultural meaning of acne is deeply entangled with normative aesthetics that reflect race, class, gender, and neoliberal purity politics. Dermatological visibility often intersects with whiteness as the presumed standard of purity, cleanliness, and moral health. In beauty discourse, "clear skin" signifies not only beauty but also self-discipline, wealth, and whiteness – an unmarked ideal. The "melancholy of whiteness" often hinges on the visible failure to embody its fantasy of cleanliness. Chinaski's acne marks him as abject, not just because it is dermatological, but because it signifies class-based lack – poverty, poor nutrition, and a lack of access to medical care. His rejection by women, teachers, and peers is filtered through this embodied stigma. Masculinity in Bukowski's world is linked to dominance and defiance, but Chinaski's grotesque flesh disrupts these expectations, rendering him fragile and feminized, unsettling the expected performance of male embodiment.

This instability solidifies Heather Widdows critique of the rise of the "beauty imperative" where deviations from bodily norms become a moral and economic liability (20–21). The face transcends the physical into the performative and acne disrupts this performance creating an incoherence between the self that is and the self that should be. This incoherence leads to a feedback loop of shame (Scheff 90–98) where the failure to meet societal standards reinforces social withdrawal, and social withdrawal further marks the individual as deficient.[5] This is mirrored in Chinaski's own avoidance and emotional withdrawal in the novel. This cycle of withdrawal and self-negation echoes Avila's reading of Chinaski's broader trajectory from self-protection to self-destruction, a pattern rooted in chronic social rejection and embodied shame (iii-iv). His refusal to attend dances, engage in romantic pursuits, or participate in public life stems not from misanthropy but from a deep awareness that his grotesque body is a failed commodity in a marketplace of appearances. Within a dermogrotesque framework, this oscillation between hypervisibility and erasure – being seen too much yet socially unseen – captures the paradox of grotesque embodiment.[6]

Within neoliberal culture, capitalism offers a solution to this failure in the form of consumable remedies: creams, scrubs, antibiotics, surgeries. The body becomes a site of endless investment, a surface to be perfected at any cost. The irony, of course, is that Chinaski's working-class background precludes such investment. He cannot afford expensive dermatological treatments or luxury cosmetics. His acne thus enacts a double marginalization – he is punished for having the condition and excluded from the means to "fix" it. As Garland-Thomson notes, the disabled figure serves as a synecdoche for the social body's dysfunction (*Extraordinary Bodies* 19; 56); and Puar critiques that this failure is not just cosmetic but biopolitical – a refusal or inability to engage in the perpetual optimization of the self (63). Chinaski's disfigured face, then, reflects the cracks in the system that is governed by poverty, exclusion, and commodified beauty standards. This dynamic is intensified by what Foucault terms the "technologies of the self" through which individuals are urged to manage, optimize, and normalize their bodies (*Technologies* 277–291). Within this framework, to have acne is to fail in one's duty to self-regulate. It marks the subject as undisciplined, uncaring, or impure. Acne becomes not only a blemish on the skin but a blemish on the moral character.

However, Bukowski refuses to write Chinaski as a willing participant in this regime. Unlike many coming-of-age protagonists who seek transformation, Chinaski remains defiantly unchanged. He does not buy into the myth of redemption through dermatological restoration. His resistance is not heroic but sullen, disillusioned, and gritty. In this way, *Ham on Rye* subverts the capitalist narrative arc in which the body must be purified, the soul uplifted, and the protagonist rendered socially acceptable. Instead, Bukowski gives us a hero whose very refusal to be cured becomes a kind of protest. This passive resistance subverts the gaze's expectations that the patient should desire and pursue normalization. Chinaski seeks treatment for his severe acne at a dermatologist early in the novel. The dermatologist prescribes him a topical treatment and urges him to avoid oily foods and to wash his face often. Chinaski, on the other hand, believes the treatment is unsuccessful, and he struggles with skin problems throughout the story. Later in the story, he consults a doctor about his boils, which are causing him great pain and discomfort. The doctor prescribes antibiotics, but Chinaski refuses to take them since he distrusts doctors and medical procedures. He eventually takes the antibiotics, and his boils heal, but he remains skeptical of the medical establishment. Bukowski portrays the medical establishment as largely ineffective and impersonal throughout the novel, with doctors more concerned with their own authority and status than with the well-being of their patients. As Lauren Berlant argues a slow death and noncompliance can themselves constitute political acts (114). In this way, Chinaski's flesh marred by acne becomes not only a symbol of exclusion but also a quiet indictment of a world that demands purity to grant value. His body, permanently *unfixable*, resists incorporation into capitalist, medical, and aesthetic economies – and in doing so, offers a muted, stubborn form of survival.

The Body as a Narrative Surface

Scholarly discussions about the body's function in narrative discourse show a basic epistemic fragmentation about its ontological status (Csordas 1–3; Weiss 10–12). While various theoretical frameworks, ranging from phenomenological embodiment to Foucauldian biopolitics debate whether the body should be read as a discursive construct, a lived experience, or a political site, a critical consensus emerges: the body transcends its biological reduction (Butler 256; Grosz 143). This view is consistent with Spinoza's monist argument that mind and body are aspects of the same substance (qtd. in Marshall 897–890) foreshadowing modern non-dualist methods in embodiment studies (Sheets-Johnstone 422–423). The same ideology is echoed in the rich Indian philosophy where the body is viewed as a complex entity, often composed of multiple layers (physical, subtle, and causal) and as a vehicle for the soul's journey (Chakrabarti 118). The body's narrative capacity, operating through what Haraway terms "situated knowledges" – material-semiotic practices mutually constituted by cultural codes ("Situated Knowledges" 590) – manifests dialectically in performative iterations of

gendered materialization (Butler 17–18), somatic archives of racialized trauma (Ahmed, "Racialized Bodies" 47–48), and posthuman reconceptualization that dissolve Cartesian binaries (Hayles 5; 21).

Beyond its materiality, the body can be understood as a spatial narrative space, where personal agency, institutional power, and meaning-making are inscribed, challenged, and rewritten (Foucault, *Birth of the clinic* 136; Butler 2). It is not static but rather a site of ongoing inscriptions by language and erasure by ideas. The body then is not merely an individual tool but a social possession and a reflection of society itself. Through discipline and control, the body is turned "docile" yet narratively charged (Foucault, *Discipline and Punish* 136; 138). It becomes a site where the "power and problems of reference" manifest themselves, where narratives shape bodies and bodies disrupt narratives (Punday 118–119). This narration and disruption process is a repeated act on the body that leads to identity formation/crisis.

Susan Bordo's *Unbearable Weight* exposed how patriarchal and capitalist forces discipline women's bodies through norms of beauty and health (21). Hortense Spillers further demonstrated how racialized bodies bear the marks of historical violence, distinguishing between "*flesh*" – brute materiality under slavery and "*body*" – a contested site of identity (67). Luna Dolezal further examined how visible bodily differences become a trifecta: source, receiver, and embodiment – of shame, and thus birth an internalized stigma, which manifests as an automatic and repetitive act of internalization (xv). Contemporary scholars argue that bodies internalize these meanings also through relational encounters with space, power, violence, technology (Ahmed, *Living a feminist life* 22; Puar xiv), state governed frameworks of disability and debility (Puar xvi), historical (Fassin 283) and colonial (Boehmer 129–137) violence and resistance, digital and pharmacological spaces (Hayles 289; Preciado 146) where organic and synthetic narratives merge (Haraway, *Simians, cyborgs, and women* 203). Bodies that resist normative temporality and spatiality, like the disabled body, perform in the generative narrative space (Garland-Thomson, *Extraordinary Bodies* 130; Kafer 10; 27; McRuer 30–32).

Thus, reading the body as an unstable yet potent narrative surface, reveals its dual role as a product of coercive systems and a site of generative resistance. However, if the body is the contested narrative space, then illness, as a disruption of embodied norms, intensifies this contestation. It is in such intensified instabilities that Chinaski's severe acne become crucial narrative sites.

Acne as an Illness Narrative

It becomes necessary to contextualize acne as an illness narrative whose cultural and affective dimensions highlight the stakes of Bukowski's text. Illness, unlike *disease* (the clinical pathology), is a *lived disruption* that encompasses the subjective, cultural, and political dimensions of suffering (Sontag 3; Carel 65–66). As per Arthur Frank, illness is a way of being, the loss of the "destination and map" that destabilizes perceived norms of health, productivity, and identity (1). He furthers the discourse by stating that illness is not just a condition to be treated; it is a story demanding witness. Stories that can repair the damage, are acts of citizenship in a community of suffering (Frank, *At the will of the body*, 81) which can suture the rupture and defy the isolation that illness imposes.

The fundamental nature of illness is as a lived phenomenon, as the disruption of the "body-self unity" (Greenlagh & Hurwitz 48–50) wherein even seemingly minor conditions impact an individual's ongoing narrative of self (Bury 267; Meril Squier 4). These lived experiences do not exist in vacuum. What is lived, later becomes what is told. The act of telling then, is not just passive recalling of the facts; it is an active process of selection, interpretation, and structuring, all of which are integral to the formation of narratives. The framing of our telling can shape our remembrance and understanding of those experiences, where the *story* becomes intertwined with the *event*. There is plurality in different *tellings* of the same event, of the same disease. What gets told, is what becomes real in the shared community. What gets told and retold within a community becomes a *shared narrative*. The told version then is a filtered representation of what was lived, which as Angela Woods concerns, could be an overinflated or a downplayed version. However, everyone's experience with illness, acute or chronic, mild or severe, is inherently

a story of adaptation, resilience, and the search for meaning (Martino & Freda 623). Questions like "*why me*?" or "*what did I do to deserve this*" (which are also prevalent in *Ham on Rye*) become a quest (as Frank would label) wherein a lot of adaptations of multiple stories (or episodes as Strawson would label) exist on the same plane. The assertion that certain illnesses are exempt from narrative consideration implies a hierarchical valuation of suffering, wherein only the experiences of those with life-threatening or debilitating diseases are deemed worthy of exploration.

Acne permeates beneath the skin – our surface and our limit (Benthien 36). It designs a story of worthlessness and ambiguity (Dalgard et al. 746; Magin et al., "Psychological sequelae of acne" 979) in social structure where beauty is a symbolic capital. This story not only leads to dehumanization of self but also of others who watch the sufferer. This act of "*watching*" is distinct from "*witnessing*", where "watching" implies distance, objectification, and aesthetic judgment; "witnessing" demands proximity, recognition, and ethical engagement. To witness someone with acne is to acknowledge the affective labor of their suffering, to understand the internalized story about shame and imperfection. However, "watching" leads to a *dual inscription of narratives*: of the *sufferer* and of the *watcher*. This inscription and its embodiment occur through the skin of the sufferer where aesthetic, racial, and diagnostic hierarchies intersect (Pinder 19), and the acne-affected subject becomes a surface to be corrected rather than a self to be encountered.

Research highlights how acne profoundly impacts adolescent identity formation, self-esteem, and social integration, despite often being dismissed as trivial (McNiven 284–86). Acne, within Western biopolitics, often metamorphizes into a moral indictment reflecting failure of personal hygiene, self-control, and aesthetic discipline. However, non-western perspectives on acne like Ayurveda and traditional Chinese medicine move beyond the localized pathology by emphasizing the psychosomatic unity of skin, mind, and environment (Baroni et al. 32–38; Kumar et al. 111–122; Ghosh et al. 1–9). Recent empirical studies further confirm that acne is not just an aesthetic inconvenience but a significant driver of psychosocial distress, depression, and social withdrawal among young adults. Its portrayal in media as a cosmetic flaw amidst the westernized appearance culture further deepens the inscriptions of its narrative (Croley et al. 559; Jones 243; Adkins et al. 7). Together, these perspectives position acne as a legitimate subject of illness narrative, deeply embedded in the politics of body aesthetics, social belonging, and medicalized selfhood.

Beyond Cure

For one thing, Bukowski's Ham on Rye does not offer a redemptive arc for Chinaski's acne. Curiously enough, it instead weaponizes his dermatological suffering as a visceral critique of medical, aesthetic and capitalistic regimes. The dermogrotesque as a conceptual lens exposes how the skin becomes a contested site where social hierarchies and capitalist mechanisms are inscribed and internalized. The key contribution of this study lies in: (1) establishing Bukowski's *Ham on Rye* as an illness narrative; (2) its call for an expanded and more inclusive framework within Medical Humanities-one that recognizes minor or stigmatized conditions like acne as critical illness narratives with profound political and literary weight and, (3) introducing the term **dermogrotesque** to ongoing discourses on the politics of the skin, aesthetic normativity, and embodied resistance. This study aims to open new avenues for medical humanities, health humanities, disability studies, and aesthetic studies by advocating for an ethical engagement with non-terminal, non-heroic illness narratives. It pushes against biomedical and capitalist hierarchies that privilege certain sufferings over others and challenges how even bodies marked by "minor" disfigurements are socially, culturally, and narratively erased. Future scholarship can expand this inquiry and apply dermogrotesque by examining acne or other forms of aesthetic disability across multiple literary traditions and works, historical periods, visual culture, social media/contemporary culture, and global contexts. Comparative studies exploring non-western narratives of visible illness, or empirical studies into how contemporary media represents dermatological difference, could further enrich and complicate the critical medical humanities landscape.

To read acne as dermogrotesque is not to pathologize it further but to witness its story - one of shame, surveillance, and unyielding visibility. Reading such stories with empathy begins the process of cultural healing and fosters a more inclusive and equitable understanding of embodiment.

Notes

1. This angst is also prevalent in some of Bukowski's poetry. In one of his poems titled "a threat to my immortality," he mentions (the girlfriend, in this case) his own appearance: "You 've got warts and scars and/all kinds of things all over/you. I do believe you're the ugliest/old man/I 've ever seen."
2. All page numbers refer to Bukowski, Charles. *Ham on rye: a novel*. Canongate Books, 2001
3. The association between bandaging, concealment, and an amplified sense of the grotesque has a long cultural lineage. Figures such as Dr. Griffin in The Invisible Man, Almasy in The English Patient, The Mummy (franchise) and numerous contemporary horror and superhero narratives (Deadpool, for example) exemplify how covering the skin paradoxically intensifies bodily ambiguity and estrangement. The bandages move beyond their medical usage into an aesthetic tradition where they heighten mystery, vulnerability, or the grotesque visibility.
4. Contrast this with Barfly (1987) whose screenplay was written by Bukowski himself. The movie transforms Chinaski from the acne-scarred, socially marginalized figure of Hamon Rye into a conventionally attractive, smooth-skinned Mickey Rourke. Because Bukowski himself wrote the screenplay, this shift may reflect both cinematic norms and a personal fantasy of self-revision, projecting an idealized, more sexually potent Chinaski. Rourke's later cosmetic surgery, producing a dermogrotesque appearance, ironically underscores the instability of aesthetic ideals.
5. Recent studies have shown that media and social platforms such as TikTok not only disseminate acne-related content of questionable quality but also reinforce unrealistic ideals of perfect skin shaping both public perception and treatment-seeking behaviors (Magin, et al. 2011; Yousaf, et al. 2020; Zheng, David X., et al. 2021).
6. Comparable representations of visible difference, grotesque embodiment, and the stigmatized skin surface appear across visual culture. David Lynch's The Elephant Man (1980) foregrounds the politics of staring and the ethics of witnessing; Peter Bogdanovich's Mask (1985) similarly examines adolescent identity formation under the pressure of hypervisibility; David Cronenberg's The Fly (1986) presents the deteriorating skin as a site of abjection and existential disintegration; and Pedro Almodóvar's The Skin I Live In (2011) reconceptualizes skin as a technologized, violable surface. Together, these films illuminate how the marked or excessive body becomes a terrain of surveillance, moral judgment, and aesthetic anxiety – precisely the dynamics that structure Chinaski's dermogrotesque embodiment.

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